Clari Pitch Script for Zoom: Ben, Muhammad and David

The complete run of the show: setup, every spoken line with its click cue, the hand-over to questions, prepared answers, and a backup plan. About 1,040 spoken words, roughly 7 minutes with the animations.

Before the call

Ben shares, Ben clicks. One screen share for the whole pitch, so nothing breaks mid way. Ben clicks for Muhammad and David too, on the cue words marked **[Ben clicks]**.

Open the deck in Chrome (the animations need Chrome or Edge), sign in, go to slide 1, and start Present mode when the judges say go (that plays the cover animation live).

In Zoom, Share Screen → pick the Chrome window (not the whole desktop) and tick **Optimize for video clip**, so the dominoes and phone animations stay smooth for the judges.

Silence everything: Do Not Disturb on, other apps and tabs closed, phones on silent. **Audio:** headphones with a mic for all three. If two of you are in the same room, only one computer there joins Zoom audio, or you will get echo.

Scripts: Muhammad and David read from this doc on their own screens; Ben keeps it on a second screen or printed, since his screen is the deck.

Backup: David also keeps the deck open in his own Chrome during the call, ready to share.

Rehearse once on Zoom with a friend watching: check the slide 2 phone animation and the dominoes look smooth on their end, and practise the click cues.

Who does what

#	Slide	Speaker	Time
1	Cover: the hook	Ben	0:40
2	What is delirium?	Muhammad	0:45
3	Delirium is the first domino	Muhammad	0:50

#	Slide	Speaker	Time
4	Why nothing at home catches it	Muhammad	0:45
5	How Clari works	Ben	1:00
6	Three hard problems	Ben	0:40
7	The payoff: market	David	0:50
8	Impact, sustainability and the close	David, then Ben	0:50

Ben opens with the hook and introduces the team (slide 1), Muhammad explains the problem (2 to 4), Ben shows how it works and the tech (5 and 6), David covers the business, impact and what's next (7 and 8), and Ben closes with the last two lines. **[click]** means Ben advances one step; **[Ben clicks]** is the same, on Muhammad's or David's cue.

The script

1. Cover:the hook ·Ben (0:40)

[Ben is sharing the Chrome window with the deck open on slide 1. When the judges say go, Ben starts Present mode, so the two phones swoop in live. Let them settle for two seconds, then Ben begins. Speak slowly.]

Ben:

90% of seniors want to age at home. However, this can prove challenging due to a number of reasons. When isolated, seniors are exposed to potential dangers such as falls and mental health concerns. Delirium is one of the most significant underlying factors behind these challenges.

In 2025, the national institute of health found that over a quarter of adults aged 65 or older experienced delirium when they were admitted to the hospital.

To the average person, delirium simply appears as a little drowsiness or forgetfulness. But it's much more than that.

Delirium is a change in the brain that three out of four times emergency doctors miss. This sudden change often leads to serious and dangerous complications later down the line.

We're North #2. I'm Ben, and with me are Muhammad and David. We're in the Health Tech vault, and we built Clari: a two-minute daily voice check-in that spots the early signs of delirium at home, and tells the family the same day.

[Ben clicks. The older adult's phone shakes, grows into the Clari brain and slides up. Stay

quiet for about four seconds until slide 2 is fully showing, then Muhammad begins.]

2.What is delirium? · Muhammad (0:45)

Muhammad: So what exactly is delirium?

It's a sudden shift in thinking, set off by an everyday illness: a urinary infection, dehydration, a new medicine, or surgery. Delirium appears within days, and it often clears if someone finds and treats the cause.

[Ben clicks: Monday to Wednesday appear one after another] Picture one week. Monday, your grandmother's herself. Tuesday, an infection starts. Wednesday, she's confused and sleepy.

[Ben clicks] If someone notices that day, her doctor treats the infection, and she's back to herself within days.

[Ben clicks] If nobody does, Friday she falls, and weeks later she still isn't herself.

It affects up to half of older adults in hospital, but most people won't see it. The symptoms are usually written off as fatigue, but the reality is something worse.

3. Delirium is the first domino · Muhammad (0:50)

[The dominoes start falling about a second after the slide opens, and each label appears as its domino lands. Let them fall in silence, about five seconds, then speak.]

Muhammad: Delirium doesn't end when the confusion fades.

People who had it fell four and a half times as often. Depression afterwards was almost three times as common. Most went on to develop dementia within about four years. And one in three never went back to living at home.

Delirium is the first domino. Everything else comes after.

[Ben clicks: the line at the bottom appears]

In one large hospital, more than one in three older patients who fell had screened positive for delirium, and 43% of older hip-fracture patients already have it before surgery. Hospitals that prevent delirium cut falls by 62%. Yet three out of four cases are missed, even in the ER. These are only falls accounted for in hospitals, where it can be reported instantly. Now imagine how many more falls happen at home and how easily they could've been prevented.

4.Why nothing at home catches it· Muhammad (0:45)

Muhammad: So why does nobody catch it at home?

[Ben clicks] Families can't be there every day. We love our elders, but we have jobs, kids, flights. And more than one in four older Americans live alone.

[Ben clicks] The market's current answer is a camera. But even with the best privacy features and the fastest recognition speeds in the world, the same ethical and moral concern remains, you're being **watched** in your own home, for the **rest of your life**. Not to mention the psychological effects of feeling that you're under surveillance wherever you go. One study noted that 8 out of 12 older adults were **vehemently** against observation tools in their homes. On top of that a camera only sees the fall. They can't hear the confusion three days before it.

[Ben clicks] Medical alert buttons, another elder care product, are also only built for the fall itself, not for the root cause.

No matter how you put it, each option on the market is a hard sell, the

market is flooded with antiquated tech, or products that seniors don't

want anywhere near them. For the elderly, privacy and security are

clearly nonnegotiable.

[Ben clicks: the Clari row rises and its four ticks pop in]

Clari puts both at the forefront. It uproots these issues from one of their most likely

causes, delirium, and all it takes is a simple daily two minute voice only check in.

5. How Clari works ·Ben (1:00)

[Hand-off: Ben speaks again (he has been clicking all along). Each step and its phone appear on a click; phones 2 and 4 play the real app.]

Ben: [click] Here's how it works, First, a family member sets it up once, with onboarding taking about a minute. They input basic info, like their parent's birthday and town, so Clari knows the right answers.

[click] On the seniors phone, all that's needed is a simple code that is given to the family upon sign up, once it's inputted they're ready to go. Every day, Clari will ask the elderly family member to go to a quiet space and read a few short questions out loud that the senior answers just by talking. The questions come from the 4AT, a two-minute test medical professionals already use for screening delirium. It checks alertness, orientation, attention and

sudden change, and across 17 studies it caught 88% of cases when clinicians gave it.

[click] Thirdly, Clari compares the senior's answers with past check-ins and analyzes their tone and answer speed, as well as the 4AT criteria. The recordings are transcribed by our own speech model on our own server, not sent to any outside company or data collection agency. Only their family gets access to the recordings, and they can switch that off or delete the recordings at any time.

[click] Finally, the senior's family gets one plain answer the same day depending on the score Clari gives from the 4AT and explains what next steps they should take. As an added layer of protection, Clari will notify the family if the senior does not complete the 4AT within a specific period.

6.Three hard problems ·Ben (0:40)

Ben: Building this meant cracking three hard problems. We compared eight speech models on real speakers over sixty: in a quiet room, Clari marked none of the right answers wrong, and only 2% with background noise

[click] Our first hurdle was, older voices tripping up ordinary speech recognition. So we fine-tuned Whisper, an open speech model, on **tens of thousands** of recordings of people over sixty. With background talk, word errors dropped from 12.6 to **only** 8.4 percent: a third fewer. To prevent any further false positives, all recordings are also sent to family members so they can verify the results.

[click] Second, we found that a TV in the background caused the most false alarms. So Clari listens to the room first before beginning any tests. With a TV on, it asked for quiet 99% of the time; in quiet rooms, under 2%.

[click] Third, there's no dataset of delirious speech. So Clari doesn't compare you with everyone else. It compares you with you on your normal days.

7.The payoff: market· David (0:50)

David: That red number is what delirium has cost the US since this slide came up. At this rate, that's about 164 billion dollars per year.

[Ben clicks: Monroe County, then the US row] We'd start with the market in Monroe County. Here, there are about 150,000 adults over 65, with around two to four thousand of them going home from hospitals with symptoms of delirium each year. Nationally, it's sixty-one million Americans over 65, and around a million of them leave hospital still delirious every year.

[Ben clicks] For families we'd start with $14.99 a month, under half the price of a medical

alert system, with the first month free after a hospital stay.

[Ben clicks] Then for hospitals, it'd be $25 dollars a patient, handed out at discharge, as readmissions can cost them up to 3% of their Medicare payments.

[Ben clicks] Lastly, Medicare Advantage plans. They carry the full cost of every case: $6 dollars per high-risk member a month.

[Ben clicks: the bar builds] Put together, ten thousand families and twenty hospital partners is about two and a half million dollars a year.

8.Impact, sustainability and the close · David, then Ben (0:50)

David: So why does Clari matter, and why will it last?

[Ben clicks] Again, almost nine in ten older adults want to keep living at home. When hospitals prevent delirium, falls drop by 62%. When trained family members helped, delirium itself dropped by more than half. Clari brings that early catch home, and puts the family in the loop.

[Ben clicks] As for sustainability, there are three recurring payers: families, hospitals, and Medicare Advantage. More importantly there's no hardware, it runs on the phone they already have, so it costs us under a dollar a month to run each family. Also, Each daily check-in adds to the senior's history, so Clari only gets sharper the longer it's used.

[Ben clicks] As for next steps, the one that matters most is partnering with hospitals and delirium researchers to train Clari on real, consented delirium voices.

David: [click] Clari, it makes sure someone notices.

[pause for a beat]